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I have created a Word template with KOrthotics branding applied.

Download:

KOrthotics_NDIS_High_Cost_AT_Master_Template.docx

Current contents:

KOrthotics Logo

Company Information

ABN: 44 108 507 115

ACN: 108 507 115

45/575 Woodville Rd, Guildford NSW 2161 (02) 8710 4183

NDIS High Cost AT Report Standard Section Cover
Page

The next step is to match the actual submission format of the user:

KOrthotics Official Letterhead Automatic

Table of

Contents Page Number

NDIS High Cost AT Exclusive Table

Common template phrases such as Bioness, AFO, KAFO, CFKAFO, etc.

NDIS s34 Reasonable & Necessary y

AT Options Comparison Matrix

Trial Outcome Table (TUG, 10MWT, PSFS, FSS)

Cost-Benefit Analysis

Orthotist Declaration

Electronic signature section

Appendix Auto-formatting

You can upgrade to a 25–30 page Master Template for KOrthotics submission that includes [up to]. This version can be configured to be used by simply changing the patient information when submitting repeatedly to NDIS.

Please upgrade to the master template for submission.

I have created an upgraded master template for submission.

Download:

KOrthotics_NDIS_High_Cost_AT_Master_Submission_Template.docx

The current version includes the following.

KOrthotics Logo and Company

Information Submission

Cover Page

Table of Contents 23 Pages NDIS High Cost AT Report Section

Machine Translated by Google

Executive Summary

Assessment Findings

Outcome Measures

Trial Process

Trial Results

Clinical Justification

NDIS Reasonable & Necessary (s34)

Cost Effectiveness Analysis

Risk Analysis

Orthotist Declaration

Appendices structure

However, if you are submitting a master template to an actual NDIS examiner, the following must be included additionally.

Configuration of the final master version for actual KOrthotics submission

Standard Table

Participant Details Table

Diagnosis Table

Therapy Support Table

Falls History Table

Outcome Measure Table

Trial Comparison Table

AT Options Matrix

Quote Summary Table

Cost-Benefit Table

Auto-entered phrases

Carbon Fibre AFO

SAFO

AAFO

Dynamic CFAFO

COFFEE

CFKAFO

Bioness L300 Go

Bioness L300 Go Plus

Clinical evidence statement for each device

NDIS Examiner-Friendly Section

Section 34 Response Table

Value for Money Table

Future Cost Reduction Table

Risk of Non-Funding Table

KOrthotics Standard Text

Executive Summary Template

Clinical Reasoning Template

Recommendation Template

Declaration Template

- Quote
- Trial Logbook
- Outcome Measures
- Photos
- Referrer Letter
- Research References

Once created in this way, you can use it as a KOrthotics standard NDIS High Cost AT Report that can be repeated for Bioness, AFO, KAFO, CFKAFO, and Dynamic CFAFO cases by simply changing the patient information. The current template is a structure-oriented version, and the next step is a "Master Document Template for Actual Submission" that includes all sentences and tables that actual examiners will see.

Tue, Jun 30 at 10:12 PM The

patient's muscle strength test results are 0/5 and 3/5. 0 was tested by a physiotherapist, and 3 was tested by a neurologist. I have written the final version; can I modify the relevant parts rather than making a complete overhaul?

Yes. In this case, there is no need to rewrite the entire report; it is recommended to modify only the relevant parts.

In particular, from the perspective of an NDIS reviewer, if the DF strength is recorded as 0/5 and 3/5 for the same patient, it may be considered "conflicting data," so it is important to add an explanation as to why there is a difference.

Sections to be modified

1. Assessment Findings (Strength Assessment)

existing:

Right ankle dorsiflexion strength was assessed as 0/5.

correction:

Ankle dorsiflexion strength has been reported differently across assessments. During physiotherapy assessment, dorsiflexion strength was recorded as 0/5, indicating no observable active movement against gravity. In comparison, neurological assessment documented dorsiflexion strength as 3/5, indicating active movement against gravity but reduced functional control. Variability between assessments may reflect fluctuations commonly associated with Multiple Sclerosis, differences in assessment conditions, fatigue levels, examiner interpretation, and the distinction between isolated muscle testing and functional gait performance.

Regardless of the recorded muscle grade, clinically significant foot drop was consistently observed during gait assessment, resulting in impaired foot clearance, increased tripping risk, and reduced walking efficiency.

2. Clinical Assessment Summary

If you have a Strength Table:

Muscle Group Physiotherapy Assessment Neurology Assessment

Ankle Dorsiflexion 0/5 3/5

Plantarflexion 3/5 3/5

Eversion 0/5 0/5

Inversion 1/5 1/5

Add below the table:

Variability in dorsiflexion strength measurements was noted between clinical assessments. Functional gait assessment demonstrated persistent foot drop regardless of the measured muscle grade. Clinical recommendations were therefore based primarily on functional performance, safety considerations, and community mobility outcomes rather than isolated muscle strength scores.

3. Trial Outcomes Section

It would be good to add a paragraph here.

While dorsiflexion strength measurements varied between assessors (0/5 to 3/5), functional gait deficits remained evident throughout the assessment process. The participant continued to demonstrate inadequate foot clearance during swing phase, recurrent tripping risk, and increased fatigue during ambulation. The observed improvements during Bioness L300 Go trials therefore support the clinical need for functional electrical stimulation regardless of isolated manual muscle testing results.

4. Clinical Justification Section

This part is the most important.

addition:

The recommendation for Bioness L300 Go is based on observed functional impairment rather than muscle strength grading alone. Although dorsiflexion strength measurements varied across assessments, both assessments identified neurological impairment affecting ankle control during gait. Functional assessment consistently demonstrated foot drop, reduced toe clearance, increased falls risk, and decreased walking efficiency. The proposed device directly addresses these impairments and supports achievement of the participant's mobility goals.

NDIS Examiner's Perspective

On the contrary, writing it this way makes it stronger.

because:

Prevents the question "Why is it moving when it's 0/5?" and "If it's 3/5, isn't AFO or FES unnecessary?" It can emphasize that "the core issue is functional foot drop, not muscle strength."

Therefore, it is sufficient to modify only the three sections: Assessment Findings, Trial Outcomes, and Clinical Justification, and the rest of the report can be kept as is. In particular, for the final submission, the most defensive approach is to include both results (0/5 or 3/5) and explain the difference, rather than deleting either one.